

“The Strictest Parents Raise the Sneakiest Kids”

What young men told us about drugs, addiction and shame in Rochdale (session 2)

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Venue: Bilal Community Centre, Rochdale

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INTRODUCTION

This report presents findings from a focus group in Rochdale in which eight young South Asian men discussed addiction, family life, faith and the response of local institutions. They described changing patterns of drug use, including large nitrous oxide canisters, discreet vaping, cannabis products resembling sweets and dealing through private social-media accounts.

The conversation moved beyond individual substances. Participants considered why people begin using drugs, why others stay away, how punishment and shame affect disclosure, and what families, schools, mosques and services could do differently. Their contributions showed considerable awareness, concern and a clear desire for earlier, more honest and more consistent action.



KEY FINDINGS

Drug markets have changed.

Larger nitrous canisters, discreet vapes and social-media dealing are increasingly common.

Mosques can welcome or exclude.

Compassion supports recovery; humiliation drives people away.

Risk feels normal. Young people regularly encounter open drug use and dealing.

Faith can support recovery.

Prayer offers hope but cannot replace professional help.

Families rarely talk. Fear and punishment can push problems underground.

Education lacks depth. Young people want current, honest and experience-led teaching.

Shame delays help. Izzat, gossip and judgement encourage silence.

Local action needs unity.

Organisational competition and poor follow-up weaken the community response.

METHODOLOGY

We held a qualitative community focus group with eight South Asian participants aged 14 to 34. Waffa Hameed and Jamal Aziz facilitated the session, drawing on lived experience, youth work and mental-health knowledge. Participants gave consent for the discussion to be recorded. Images of different substances, short videos and agree-or-disagree statements were used to introduce the subject and encourage discussion. Participants then responded to questions and to one another, sharing observations, experiences and different points of view. All contributions in this report have been anonymised. The recording was transcribed and reviewed to identify recurring themes before being deleted after the report was completed.

CONTINUE FOR DETAILED ANALYSIS >>>

HOW THE YOUNG MEN UNDERSTOOD ADDICTION

Participants did not agree on a single definition of addiction. Most believed that trying a substance may begin as a choice, but repeated use can develop into dependency and make stopping increasingly difficult. Some emphasised personal responsibility and willpower, noting that people exposed to the same pressures do not always make the same decisions. Others focused more on environment, peer influence and the normalisation of drug use within friendship groups and neighbourhoods.

One participant illustrated this through two friends visiting the same park: one went to play football, while the other went to smoke cannabis. Participants also connected addiction with bereavement, anxiety, boredom and the desire to escape difficult feelings. Overall, they understood it as a complex interaction between personal choices, individual circumstances and the surrounding environment—not simply as weakness or bad character.

1. CHANGING SUBSTANCES, SOCIAL MEDIA AND THE NORMALISATION OF RISK

Participants described a drug market that had changed significantly in its scale, appearance and accessibility. Older participants remembered nitrous oxide being sold in small silver bulbs. Much larger canisters were now openly available from some shops for approximately £10 to £25. They described groups using nitrous oxide inside cars and young men driving while inhaling from balloons, creating serious risks for both the driver and other road users. “I’ll see boys casually driving with a balloon in their mouth.”

Vaping was also described as widespread and easy to conceal because it did not leave the smell associated with cigarettes or cannabis. Participants knew of young people who believed they were accepting an ordinary vape but were instead exposed to products containing concentrated cannabis or spice. Cannabis edibles created another concern because their colourful packaging could make them look like ordinary sweets.

Social media had also changed how drugs were promoted and sold. Participants described dealers using private Snapchat accounts to display complete menus, sometimes turning images upside down in an attempt to avoid automated detection. They also discussed musicians and online personalities who presented drugs, money and the “high life” as attractive or entertaining.

The youngest participant, aged 14, said that seeing drug dealing openly had begun to feel “normal” because it happened so frequently. Other participants were struck by the difference between what younger people now witnessed and what older generations had seen at the same age.

Participants did not describe every person involved in dealing as simply a “bad lad.” One connected dealing with deprivation, visible examples of quick money and the belief that it offered a way out of poverty. This did not excuse the harm caused, but it showed why enforcement alone would not address every reason young people became involved.

Prevention must therefore keep pace with changing substances and methods of selling. It must also reach the physical and online spaces in which young people are encountering these risks.

2. PARENTS, SILENCE AND THE PARALLEL LIVES OF TEENAGERS

Most participants said that they had not discussed drugs or addiction openly with their families. This did not necessarily mean that parents were unconcerned. Long working hours, school, college and mosque classes could leave families with little time for meaningful conversations. Parents might also lack the knowledge or confidence to raise the subject.

Participants believed that when drug use was discovered, the likely response in some families would be scolding, threats or physical punishment. They questioned whether these reactions would stop the behaviour or simply teach a young person to hide it more carefully. A phrase introduced during the discussion captured this concern:

“The strictest parents raise the sneakiest kids.”

Participants nevertheless recognised that parents needed to establish boundaries. Their concern was not with parents setting limits, but with fear and punishment becoming the only response.

They also remembered conversations and stories that had stayed with them. One participant recalled finding a discarded nitrous oxide bulb as a child. His father told him to throw it away and explained what it was. Another remembered his father warning him about the influence of friends:

“If you are in a group of five drunks, you become the sixth. If you are in a group of five pious, praying people, you become the sixth.”

These examples showed the value of beginning conversations before a crisis develops. Parents need accurate information, but they also need practical support to listen, set boundaries and respond without immediately closing down communication.

3. SHAME, IZZAT AND SUFFERING IN SILENCE

Participants repeatedly returned to izzat, meaning family honour or reputation, and sharam, meaning shame or embarrassment. Within a close-knit community, one person's addiction could quickly become the subject of gossip about the whole family. Fear of this judgement could influence whether a problem was acknowledged and whether professional help was sought.

Participants described how families might quietly provide money to a relative using drugs because they feared that the person would otherwise steal, become involved in crime or expose the problem publicly. Others spoke about families denying addiction or explaining severe mental distress through jadoo, meaning black magic, or through the influence of spiritual forces.

One example involved a family attending an NHS appointment while also planning to take their son to Pakistan for spiritual treatment. Participants did not dismiss faith or spiritual support. Their concern was that relying on these explanations alone could delay proper assessment when someone was experiencing addiction, withdrawal, psychosis or another serious mental-health problem.

Shame could also become internal. Participants described how a person might begin to think of himself as weak or worthless, creating a private commentary in his head that made his mental health worse. Someone known publicly as a “good kid” might find it especially difficult to admit that he was struggling because of the effect this could have on his own reputation and that of his family.

Challenging this silence does not require attacking culture or faith. It requires a different understanding of honour: one in which seeking help, protecting someone from further harm and supporting recovery are responsible actions rather than causes for shame.

4. MOSQUES: PREVENTION, SUPPORT AND RECOVERY

The mosque occupied a complicated but important place in the discussion. Participants recognised that some mosques and imams were willing to address addiction and social problems, while others lacked the knowledge, confidence or resources to respond. They also distinguished between the compassion taught by faith and cultural attitudes that could drive struggling people away.

One participant recalled an elder publicly challenging a young man for wearing an earring when he entered the mosque. The embarrassment caused him to leave. Participants felt that welcoming him, allowing him to pray and then speaking to him privately would have been more compassionate and more likely to keep him connected.

There were also powerful examples of mosques and faith supporting recovery. One participant described encouraging young men who used drugs to return to the mosque, sometimes teaching them how to pray again after years away. Some had asked whether they were even allowed to enter the mosque because of what they had taken. He believed that continuing to attend and trying to change was better than feeling completely excluded.

Another participant recalled an imam giving a compassionate funeral address for a young man associated with drugs and crime. The speech moved the young man's friends because the imam acknowledged his humanity rather than reducing his entire life to his mistakes. The same imam had criticised harsh treatment within some mosques and the way humiliation could push young people towards the streets.

Waffa Hameed, one of the facilitators, shared how returning to prayer had become an important foundation in his recovery from alcohol dependency and in rebuilding his relationships with his wife and children. His experience also showed that faith formed part of a wider recovery journey rather than replacing professional or practical support.

Participants felt that occasional talks from visiting speakers were not enough. They wanted addiction and substance misuse to become a regular part of preventative education, potentially once or twice each month. Muslim children may spend around two hours a day, five days a week, attending mosque or madrassah. Participants saw this as a major opportunity to reach children before harmful behaviour begins. They also believed that when one mosque demonstrated effective work, others were more likely to follow.

Imams should have basic awareness of addiction and know where people can obtain professional help. However, their main responsibility is religious instruction and the running of the mosque. They cannot reasonably be expected to become specialist addiction workers as well.

Participants also noted how rare it was to encounter Muslim or South Asian people working within recovery services. Having access to somebody who understood their family, faith and community context was seen as valuable, particularly when a person was already afraid of being judged.

From The Salik Project UK's perspective, the wider opportunity is to identify and support trusted people already present within mosque and grassroots networks. With appropriate training, payment and support from recovery organisations and Public Health, these community connectors could lead regular awareness work and help families reach professional services.

5. WHERE SCHOOLS AND MOSQUES ARE FALLING SHORT

Participants did not believe that schools or mosques provided enough meaningful education about drugs and addiction. One person who had left school five years earlier could not remember a substantial session on the subject. A younger participant recalled addiction being mentioned in PSHE, but without the depth or openness of the focus group.

Their criticism was also about how information was delivered. Participants felt that formal lessons could be general, unenthusiastic and disconnected from the substances and pressures young people were actually encountering.

Jamal Aziz, one of the facilitators, recalled an earlier community-centre event led by a person with experience of addiction and crime. The venue was packed, and the speaker held people's attention because his account felt real and relevant. Jamal remembered leaving the session thinking: "I've learnt more here than I've ever learnt in school."

This reflected a wider theme within the discussion: information was more powerful when it was current, interactive and connected to *real experience*.

The focus group itself used photographs of different substances, short videos and agree-or-disagree statements to encourage participation. The Salik Project UK also uses images, video, lived-experience testimony and a drug-awareness box containing examples of unfamiliar packaging and equipment. These methods allow people to ask questions, challenge assumptions and recognise items that may otherwise go unnoticed.

Participants also said that imams could complete years of religious study without receiving basic education about addiction or mental health. Awareness training could help imams recognise when religious guidance was not enough and when someone needed urgent professional support. This should help imams signpost people appropriately, not transfer responsibility for specialist addiction work onto them.

Schools and mosques have regular access to young people. The discussion showed that this opportunity is not being fully used. One-off talks are unlikely to be enough; young people need repeated opportunities to learn, ask questions and hear messages delivered in ways that feel relevant to their lives.

6. CREATIVE ENGAGEMENT AND COLLECTIVE ACTION

Participants questioned why local mosques and community organisations did not work together more closely on addiction. They saw competition for funding, recognition and status as a barrier to collective action. If organisations combined their knowledge, relationships and resources, they believed they could achieve more than any single group acting alone.

The discussion also highlighted the importance of creative engagement. The Young Khadims' Funeral Workshop was mentioned as an example of involving young people in a practical and emotionally meaningful activity. Rather than simply telling them what to think, the workshop encouraged reflection on death, accountability and the direction of their lives.

This supported the wider point that young people do not always respond to lectures or posters. Videos, images, practical objects, open discussion and lived experience can make difficult subjects more immediate and understandable. Creative delivery does not mean making light of addiction; it means finding ways to hold attention and create honest conversation.

When asked how they would help a friend struggling with addiction, participants suggested several practical responses. They spoke about keeping the person from becoming isolated, helping them stay occupied, bringing them to an event, connecting them with someone who could help and referring them to a service such as High Level Northern Trust. One participant said that he would continue taking a friend to places of support because the message might not "click" the first time.

Participants believed that responsibility was shared. Government and police had a role in addressing the supply of drugs, but families, schools, mosques, recovery services and community organisations also had a part to play in prevention and support.

They were also frustrated by consultations that appeared to produce no visible action. Communities could become disheartened when they were repeatedly asked to discuss the same problems without being told what changed as a result. Listening must therefore be followed by action, continued contact and honest feedback about what can and cannot be done.

Conclusion: What We Learned

The focus group showed that the young men already possessed considerable awareness of addiction and the pressures surrounding it. They connected substance use with peer influence, anxiety, bereavement, boredom, deprivation and attempts to escape difficult feelings. They did not excuse harmful behaviour, but neither did they reduce addiction to weakness or bad character.

Their accounts demonstrated how quickly the drug environment had changed. Large nitrous oxide canisters, discreet vapes, cannabis products resembling sweets and private social-media menus had changed how young people encountered substances. Particularly concerning was the youngest participant's description of public dealing as normal. When a risk becomes familiar, it can begin to appear ordinary.

A generational gap also ran through the discussion. Parents may care deeply but lack the time, knowledge or confidence to discuss drugs before a problem develops. Threats and punishment can make young people more secretive, while shame, izzat and fear of gossip can delay families from seeking help.

Faith was viewed as an important source of hope, belonging and recovery, but not as a complete treatment. Participants wanted mosques to offer compassion rather than humiliation and wanted imams to understand when professional support was required. At the same time, they recognised that sustained addiction work could not simply be added to an imam's existing responsibilities.

The discussion also showed that the method of engagement matters. Lived experience, images, video, practical activities and open conversation held greater value than generic information delivered without depth or enthusiasm. Young people wanted education that reflected the substances, pressures and online environments they actually encountered.

Above all, the participants wanted action rather than another conversation that disappeared. Their contributions point towards current drug education, practical support for parents, basic awareness for imams, paid grassroots leadership, positive activities, clearer routes into help and visible follow-up from the organisations asking communities to share their experiences.

RECOMMENDATIONS

The priorities below are rooted in the concerns and ideas raised by the young men. Where a specific delivery model is proposed—particularly the mosque pilot—it represents The Salik Project UK's practical recommendation for turning those findings into action.

1. Keep drug education up to date

Schools and community organisations should regularly update the information they provide about drugs. This should include large nitrous oxide canisters, vapes containing cannabis or spice, cannabis edibles, changing packaging and dealing through private social-media accounts.

A practical first step would be an annually reviewed local resource that can be used by schools, mosques, parents and youth organisations.

2. Use creative methods and lived experience

Drug education should include suitable speakers who can talk honestly about addiction, its consequences and recovery. Sessions should also use discussion, images, short videos and practical resources rather than relying entirely on lectures or written information.

These approaches should be used regularly and adapted to the age and experiences of the young people taking part.

3. Help parents begin conversations earlier

Parents should be offered practical sessions on how to talk to their children about drugs calmly and openly. This should include accurate information, listening skills, appropriate boundaries and ways to respond when they are concerned.

A short conversation guide could help parents raise the subject before a crisis rather than relying on threats or punishment after a problem has developed.

4. Give imams basic addiction awareness

Imams should receive basic awareness training to help them recognise possible addiction or serious mental distress, respond without judgement and direct people towards appropriate services.

This training should support their religious role. It should not expect imams to provide specialist addiction treatment or take responsibility for managing complex cases themselves.

5. Pilot paid community connectors in mosques

Based on the participants' call for regular mosque-based education, The Salik Project UK recommends a 12-month pilot in one or two mosques.

Trusted people should be identified through existing mosque and grassroots networks, paid for their time and given basic facilitation training. With ongoing support from recovery organisations and Public Health, each connector would deliver one addiction-awareness session a month during madrassah hours and help families find appropriate services.

The pilot should be closely monitored, looking at attendance, responses from young people and families, changes in understanding, requests for help and lessons for possible expansion. This work must be funded rather than added to the responsibilities of unpaid volunteers.

6. Fund positive activities for young people

Participants identified boredom, isolation and a lack of alternatives as factors that could draw young people towards harmful peer groups or substance use.

Public Health and local partners should support affordable sports, practical workshops, youth projects and other regular activities that give young people something constructive to join and trusted adults with whom they can build relationships.

7. Resource grassroots groups to deliver sustained work

Young people should receive clear guidance on what to do when they are worried about a friend. This should include staying in contact, reducing isolation, encouraging positive activity, connecting the person with professional support and continuing to offer help even if the person is not ready the first time.

A short phone-friendly guide should include current details of local recovery, mental-health and emergency services.

8. Tell communities what happens next

Organisations conducting focus groups and consultations should report back to the people who took part. Communities should be told what was learned, what action will follow, who is responsible and what cannot currently be delivered.

About Us & Our Collaborators

ABOUT THE SALIK PROJECT UK (ROCHDALE)



The Salik Project UK works around two connected aims: breaking the stigma surrounding addiction in South Asian communities and helping recovery services become more culturally informed and responsive to the people they serve. We do this through community education, outreach, research, lived experience and partnership working.

We are not a treatment provider and do not believe this work should belong to one organisation. We want to empower community organisations to take up the issue themselves, while working with recovery services, Public Health and other partners to strengthen understanding, improve access to support and ensure South Asian voices help shape the services intended for them.

About The Young Khadims



The Young Khadims is a youth-led volunteer and community group based in Rochdale, Greater Manchester. The group brings young people together to serve local residents, support community initiatives and strengthen a culture of compassion, responsibility and selfless service.



About High Level Northern Trust

High Level Northern Trust is a Rochdale-based charity and recovery service supporting people affected by addiction. It provides a safe, welcoming space for people in recovery, while also offering guidance and support to their families and wider support networks.

About Action Together



Action Together supports voluntary, community, faith and social enterprise organisations across Rochdale, Oldham and Tameside through funding, advice, training and partnership building. It has provided funding to The Salik Project UK, helping us deliver community-based addiction awareness, education and engagement work in Rochdale.

With Speacial Thanks to The Young Khadims Rochdale



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